CLINICAL NOTE · TFOS DEWS II DIAGNOSTIC WORK-UP & STAGED MANAGEMENT

# Dry Eye Disease Evaluation

Structured dry eye work-up in the recommended order (least to most invasive), with sub-type classification and a staged treatment plan.

Completed by: Clinician + technician | Use as: Note template | Time: 20–30 min

| Patient name | Date of birth | Patient ID / MRN | Date of service | Clinician |
|---|---|---|---|---|
| | | | | |

## 01 Symptoms & contributing factors

OSDI (EYE-07) / 100 | DEQ-5 / SPEED (if used) | Symptom duration | Worse: ○ Morning ○ Evening ○ Constant

Contributing factors

- ☐ Screen use > 4 h/day
- ☐ Contact lens wear
- ☐ Prior LASIK / PRK / cataract surgery
- ☐ Blepharoplasty / lid surgery
- ☐ Menopause / HRT
- ☐ Rosacea
- ☐ Sjögren's / autoimmune
- ☐ Thyroid eye disease
- ☐ Diabetes
- ☐ CPAP / lagophthalmos
- ☐ Low humidity / air-con
- ☐ Glaucoma drops (preserved)
- ☐ Antihistamines / antidepressants
- ☐ Isotretinoin
- ☐ Allergy
- ☐ Smoking
- ☐ Other: _____

Treatments tried & response

Current drops / frequency / preservatives

## 02 Diagnostic tests

in order

| Test | OD | OS | Abnormal if |
|---|---|---|---|
| Tear meniscus height | mm | mm | < 0.2 mm |
| Non-invasive tear break-up time (NIBUT) | s | s | < 10 s |
| Tear osmolarity | mOsm/L | mOsm/L | ≥ 308 or inter-eye difference > 8 |
| MMP-9 (InflammaDry) | ○ + ○ - | ○ + ○ - | positive |
| Fluorescein TBUT | s | s | < 10 s |
| Corneal staining (Oxford 0–5 / NEI 0–15) | | | > 5 spots |
| Conjunctival staining (lissamine green) | | | > 9 spots |
| Lid margin staining / lid wiper epitheliopathy | | | ≥ 2 mm length & ≥ 25% width |
| Schirmer I (with / without anaesthesia) | mm / 5 min | mm / 5 min | < 10 mm (no anaes.) |
| Meibography — gland dropout (meiboscore 0–3) | | | grade ≥ 1 |
| Meibomian expressibility (glands expressible, lower central) | | | reduced on practice scale |
| Meibum quality (0 clear – 3 toothpaste) | | | grade ≥ 1 |
| Lid margin: telangiectasia, notching, plugging | | | |
| Collarettes (Demodex) — grade | | | any |
| Blink: rate / incomplete / lagophthalmos | | | |
| Conjunctivochalasis / other | | | |

**TFOS DEWS II:** dry eye disease = symptoms (OSDI ≥ 13 or DEQ-5 ≥ 6) **plus at least one** homeostasis marker: NIBUT < 10 s, osmolarity ≥ 308 mOsm/L in either eye (or inter-eye difference > 8), or ocular-surface staining (> 5 corneal spots, > 9 conjunctival spots, or lid margin ≥ 2 mm length & ≥ 25% width).

## 03 Classification

**Diagnosis**

- ○ Dry eye disease confirmed
- ○ Not confirmed — consider differentials

**Sub-type**

- ○ Evaporative (MGD)
- ○ Aqueous deficient
- ○ Mixed
- ○ Neuropathic component

**Severity**

- ○ Mild
- ○ Moderate
- ○ Severe

**Associated**

- ☐ Blepharitis
- ☐ Demodex
- ☐ Allergic conjunctivitis
- ☐ Exposure
- ☐ Conjunctivochalasis

- ☐ **H04.12-** Dry eye syndrome of lacrimal gland
- ☐ **H01.0-** Blepharitis
- ☐ **H16.22-** Keratoconjunctivitis sicca, not Sjögren's
- ☐ **B88.0** Demodex (other acariasis)
- ☐ **H02.88-** Meibomian gland dysfunction (lid-specific)
- ☐ **M35.01** Sjögren's with keratoconjunctivitis

## 04 Management

staged — escalate if inadequate response

| Treatment | Selected | Details |
|---|---|---|
| Step 1 — education, environment, screen habits, lid hygiene, warm compresses, preservative-free lubricants | ☐ | |
| Step 2 — PF lubricants / gels, tea tree or lotilaner for Demodex, punctal plugs, moisture goggles | ☐ | |
| Step 2 — in-office: thermal pulsation, IPL, microblepharoexfoliation, gland expression / probing | ☐ | |
| Step 2 — Rx: cyclosporine, lifitegrast, perfluorohexyloctane, varenicline nasal spray, short-course steroid | ☐ | |
| Step 2 — oral / topical azithromycin or doxycycline (MGD / rosacea) | ☐ | |
| Step 3 — autologous serum, therapeutic / scleral contact lenses, oral secretagogues | ☐ | |
| Step 4 — amniotic membrane, surgical punctal occlusion, tarsorrhaphy; refer cornea | ☐ | |

| Medication | Dose / strength | Eye / route | Frequency | Start date | Notes / adherence |
|---|---|---|---|---|---|
| | | | | | |
| | | | | | |
| | | | | | |

**Procedure performed today (eye, settings)**

**Review in**

Retest the same markers at follow-up (OSDI, NIBUT, osmolarity, staining) to measure response.

**Clinician signature**

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**Clinician name & credentials**

**Date / time signed**